

ASHAMitra — Sign-off-pending rules

Auto-generated from `assets/data/asha_engine.json` — engine `asha_cdss_engine_v2` **v2.6.0-guideline-aligned**.

These rules carry `clinical_sign_off_pending: true`. They are **live** — they fire and affect the band exactly like any other rule — but are flagged as **draft additions awaiting a clinician's sign-off** (newer guideline items or proxy thresholds). When one fires, the pipeline attaches a `SAFETY_A_001` advisory note to the output (see `PIPELINE.md`, Layer 9). This file is the validation worklist.

Total: 64 sign-off-pending rules.

In easy words

These rules are **already switched on and working** — when one matches, the app warns the worker exactly like every other rule. The only difference: they are **newer rules a doctor hasn't formally approved ("signed off") yet**.

Think of a new staff member who is already doing the job while their paperwork waits for a senior's signature — nothing is broken, the work still happens. This page is simply the **list a doctor should review and approve**. The **Scenario** column shows a real situation in which each rule would fire.

Newborn Checkup (0–28 days) — `newborn` (12)

Rule	Type	Band	Scenario (example case — when it fires)	Action shown to worker
NB-008	hard_stop	 RED	Winter home visit to a 4-day-old: the baby feels cold to touch and isn't warming up.	PSBI hard-stop. Keep warm (skin-to-skin/kangaroo care), cover head. Refer SNCU/FRU immediately. Hypothermia is a major neonatal killer.
NB-010	hard_stop	 RED	A 10-day-old has many pus-filled blisters on the skin (or pus draining from an ear).	Many pustules/pus-filled blisters or ear pus = PSBI local bacterial infection. Refer FRU/DH immediately.
NB-011	hard_stop	 RED	The soft spot on a 2-week-old's head is bulging and tense.	Bulging fontanelle = possible meningitis. Refer SNCU/FRU immediately.
NB-COMB-002	combination	 RED	A 3-day-old looks deeply yellow AND refuses to breastfeed.	Jaundice + poor feeding = kernicterus risk. Refer SNCU immediately.
NB-COMB-003	combination	 RED	A newborn is floppy / barely moving AND is breathing hard.	Lethargy + breathing difficulty = severe sepsis. Refer SNCU immediately.
NB-COMB-004	combination	 RED	A 6-day-old has a red, pus-y belly-button AND a fever.	Umbilical infection + fever = systemic sepsis risk (PSBI). Keep the baby warm, keep breastfeeding, call 108 and take to SNCU/FRU now, and inform the ANM/MO. Do not give any medicine yourself.
NB-COMB-005	combination	 RED	A 5-day-old won't take the breast AND feels hot to touch.	Not feeding + fever = possible serious infection (PSBI). Keep the baby warm, keep trying to breastfeed, call 108 and take to SNCU/FRU now, and inform the ANM/MO. Do not give any medicine or injection yourself.
NB-VITAL-005	numeric	 RED	Thermometer reads 35.0°C on a newborn.	Temp < 35.5°C — hypothermia. Start kangaroo mother care, refer SNCU.
NB-VITAL-006	numeric	 RED	A newborn's heart rate is counted at 190/min.	HR > 180/min — severe tachycardia. Refer SNCU.

Rule	Type	Band	Scenario (example case — when it fires)	Action shown to worker
NB-VITAL-007	numeric	 YELLOW	Birth weight is 2.2 kg.	Weight 1.8–2.5 kg — LBW. KMC, breastfeed every 2 h.
NB-VITAL-008	numeric	 RED	Birth weight is 1.6 kg.	Weight < 1.8 kg — HBNC referral threshold. KMC + expressed breast milk + refer SNCU.
NB-VITAL-009	numeric	 YELLOW	A newborn's temperature is 36.0°C.	Temp 35.5–36.4°C — cold stress. KMC, warm the room, recheck in 1 hour.

Child Health Check (2 months–5 years) — child (16)

Rule	Type	Band	Scenario (example case — when it fires)	Action shown to worker
CH-009	hard_stop	 RED	A 2-year-old vomits up everything given and can't keep water down.	Vomiting everything = IMNCI general danger sign. Cannot keep anything down — refer FRU/DH immediately.
CH-010	hard_stop	 RED	A feverish 3-year-old has a stiff neck and cries at bright light.	Stiff neck/severe headache with fever = possible meningitis/encephalitis. Call 108, refer FRU/DH immediately.
CH-004	hard_stop	 RED	A 1-year-old refuses to eat or drink anything at all.	Complete refusal to eat/feed = IMNCI general danger sign. Call 108 and take to FRU now.
CH-013	hard_stop	 RED	A coughing 8-month-old's lower chest sucks inward with each breath.	Lower chest indrawing or stridor at rest = severe pneumonia / very severe disease. Keep the child calm, call 108 and take to FRU now.
CH-COMB-002	combination	 RED	A child has had fever for 6 days AND a cough.	Fever > 5 days + cough = possible pneumonia or malaria. Refer PHC today.
CH-COMB-003	combination	 RED	A child has diarrhoea AND is refusing to eat.	Diarrhoea + refusal to eat = severe dehydration risk. If the child can swallow, give ORS by spoon; call 108 and refer to FRU now.
CH-COMB-004	combination	 RED	A child has diarrhoea AND sunken eyes / dry lips.	Diarrhoea + dehydration signs = IMNCI Plan C. Refer FRU immediately for IV fluids, continue ORS en route.
CH-VITAL-004	numeric	 RED	A 9-month-old's breathing is counted at 55/min.	RR ≥ 50/min (2 mo–1 y) = severe pneumonia. Refer FRU.
CH-VITAL-005a	numeric	 RED	A child's temperature is 40.2°C.	Temp ≥ 40°C = hyperpyrexia. Refer FRU, test for malaria.
CH-VITAL-005b	numeric	 YELLOW	A child's temperature is 39.5°C.	Temp 39–40°C = high fever. Refer PHC, give paracetamol.
CH-VITAL-006	numeric	 RED	Weight-for-age is far below normal (z-score around –3.5).	WFA Z-score < -3 = severe underweight. Refer NRC.
CH-VITAL-006b	numeric	 YELLOW	Weight-for-age is mildly low (z-score around –2.4).	WFA Z-score -3 to -2 = moderate underweight (MAM). Refer ICDS/Anganwadi for supplementary nutrition, weigh monthly. (MCP card: yellow zone, pg 28–31)
CH-VITAL-007	numeric	 YELLOW	A 3-year-old is breathing 42/min.	RR ≥ 40/min (child 1–5 y) = possible pneumonia. Refer PHC within 24 h. Chest indrawing = RED.

Rule	Type	Band	Scenario (example case — when it fires)	Action shown to worker
CH-VITAL-008	numeric	 YELLOW	A finger-oximeter reads 92% on a child.	SpO2 90–94% = moderate hypoxia. Keep the child calm and upright, refer to PHC promptly.
CH-011	yellow	 YELLOW	A child's stool has visible blood.	Blood in stool = dysentery, needs antibiotics. Refer PHC within 24 h. With dehydration/lethargy = FRU.
CH-012	yellow	 YELLOW	A child's palms and inner eyelids look very pale.	Pallor = anaemia. Refer PHC for Hb. Severe pallor = FRU (possible transfusion).

Pregnant Mother Checkout (ANC) — pregnancy (21)

Rule	Type	Band	Scenario (example case — when it fires)	Action shown to worker
ANC-008	hard_stop	 RED	A pregnant woman is very pale and gets breathless climbing a few steps.	Severe anaemia is a leading maternal killer. Severe pallor/breathlessness = refer FRU/DH for Hb + transfusion planning.
ANC-010	hard_stop	 RED	At 8 months, a sudden gush of fluid leaks (waters broken early).	Sudden gush/continuous leaking = PROM, infection/preterm-labour risk. No vaginal exam, keep a clean pad, refer FRU/DH.
ANC-009R	hard_stop	 RED	A pregnant woman has a fever with shaking chills / rigors.	Fever with rigors in pregnancy = possible malaria or sepsis. Lay on left side, call 108 and take to FRU now.
ANC-COMB-003	combination	 RED	A pregnant woman is bleeding AND the baby is barely moving.	Bleeding + reduced fetal movement = placental emergency. Refer FRU immediately.
ANC-COMB-004	combination	 RED	High blood pressure AND blurred eyesight.	High BP + blurred vision = severe pre-eclampsia. Lay on left side, call 108 and take to FRU now.
ANC-COMB-005	combination	 RED	A bad headache that has lasted more than 2 days and is worsening.	Severe or persistent headache = pre-eclampsia danger sign. Refer FRU/DH immediately.
ANC-COMB-006	combination	 RED	A headache together with swelling of the face/legs.	Headache + swelling = pre-eclampsia. Refer FRU immediately.
ANC-VITAL-005	numeric	 RED	BP reads 168/100.	Systolic BP $\geq$ 160 mmHg — severe hypertension. Lay on left side, call 108 and take to FRU now.
ANC-VITAL-006	numeric	 RED	A urine dipstick shows 2+ protein.	Urine protein $\geq$ 2+ = pre-eclampsia. Refer FRU.
ANC-VITAL-007	numeric	 YELLOW	The belly measures 4+ weeks bigger/smaller than the dates suggest.	Fundal height >4 weeks discrepancy = IUGR or polyhydramnios suspicion.
ANC-VITAL-008	numeric	 RED	Labour pains start at 34 weeks (before 37).	Labour pain or leaking before 37 weeks = preterm labour. Refer FRU without cervical examination if pains/leaking present.
ANC-VITAL-009	numeric	 RED	Labour pains have continued 14 hours without delivery.	Labour pain >12 h = obstructed labour risk. Refer FRU/DH immediately.
ANC-VITAL-010	numeric	 RED	Waters broke 13 hours ago but labour hasn't started.	Leaking >12 h without labour = PROM with chorioamnionitis risk. Refer FRU now.
ANC-VITAL-011	numeric	 RED	A lab report shows TSH 12 mIU/L.	TSH > 10 mIU/L on report = thyroid problem. Refer to PHC/MO — do not start or change any medicine yourself.

Rule	Type	Band	Scenario (example case — when it fires)	Action shown to worker
ANC-VITAL-012	numeric	YELLOW	A lab report shows TSH 6 mIU/L.	TSH 2.5–10 mIU/L on report = mildly raised thyroid. Refer to PHC/MO; do not give medicine yourself.
ANC-VITAL-013	numeric	YELLOW	A 2-hour sugar test (OGTT) reads 150 mg/dL.	High sugar on report (GDM). Refer to PHC/MO for GDM management. ASHA role: counsel balanced diet and ensure follow-up. Do not give medicine or insulin yourself.
ANC-VITAL-014	numeric	RED	The baby's heartbeat is counted at 110/min.	FHR <120 bpm = fetal distress. Position mother left lateral, refer FRU/DH immediately.
ANC-VITAL-015	numeric	RED	The baby's heartbeat is counted at 170/min.	FHR >160 bpm = fetal distress (maternal fever or chorioamnionitis). Refer FRU now.
ANC-011	yellow	YELLOW	A mild headache with no other danger sign.	Headache — refer PHC within 24 h to check BP. If severe/persistent, or with swelling/blurred vision = FRU now.
ANC-012	yellow	YELLOW	A pregnant woman reports feeling dizzy and weak.	Dizziness/weakness — refer PHC for Hb + BP check (anaemia or low BP).
ANC-009	yellow	YELLOW	A fever without chills/rigors in pregnancy.	Fever in pregnancy — refer PHC within 24 h (check malaria/UTI/infection). With rigors, very high fever, or any other danger sign = FRU/DH now.

Postpartum Checkup (0–42 days) — `delivery_pnc` (10)

Rule	Type	Band	Scenario (example case — when it fires)	Action shown to worker
PNC-001	hard_stop	RED	A mother 2 days after delivery is bleeding heavily / soaking pads fast.	PPH hard-stop. Do firm external fundal massage over the uterus, lay the mother down, call 108 and take to FRU/DH now.
PNC-0065	hard_stop	RED	A new mother faints or cannot stand up.	Severe postpartum dizziness/collapse = possible concealed bleeding or shock. Lay flat with legs raised, call 108 and take to FRU now.
PNC-COMB-003	combination	RED	Postpartum bleeding AND severe weakness/dizziness.	Bleeding + weakness = hypovolaemic shock. Lay head down, refer FRU immediately.
PNC-COMB-004	combination	RED	A painful, red, swollen breast AND fever.	Breast pain + fever = breast abscess or mastitis. Refer PHC for antibiotics.
PNC-VITAL-004	numeric	RED	BP reads 145/95 after delivery.	Postpartum systolic BP ≥ 140 = postpartum pre-eclampsia. Lay on left side, call 108 and take to FRU now.
PNC-VITAL-005	numeric	YELLOW	A new mother's pulse is 118/min.	HR > 110/min = sepsis or anaemia risk. Refer PHC for evaluation.
PNC-VITAL-006	numeric	RED	2 or more pads are fully soaked within 30 minutes.	≥ 2 pads soaked in 30 min = PPH. Uterine massage, call 108.
PNC-VITAL-007	numeric	RED	More than 5 pads are soaked in a single day.	More than 5 pads per day (HBNC threshold) = excessive bleeding. PPH/sepsis risk — refer FRU now.
PNC-VITAL-008	numeric	RED	Temperature is 39.0°C after delivery.	Temp $\geq 38.9^\circ\text{C}$ postpartum = severe puerperal sepsis. Keep the mother warm and hydrated, do not give medicine yourself, call 108 and take to FRU now.

Rule	Type	Band	Scenario (example case — when it fires)	Action shown to worker
PNC-009	yellow	 YELLOW	A new mother is persistently sad, not interested in the baby, not sleeping.	Postpartum depression screen. Counsel, involve family, refer PHC/MO. Any thought of self-harm or harming the baby = urgent referral.

Immunization — UIP National Schedule — immunisation (1)

Rule	Type	Band	Scenario (example case — when it fires)	Action shown to worker
IMM-006	yellow	 YELLOW	Parents report the child had a severe reaction after the previous injection.	Severe reaction after a previous dose (AEFI). Refer to MO before the next dose — do not auto-schedule.

Emergency — Global Hard-Stop Engine — emergency (4)

Rule	Type	Band	Scenario (example case — when it fires)	Action shown to worker
EM-006	hard_stop	 RED	A person is brought in after a snakebite (or animal bite).	Hard-stop. Immobilise the limb (splint), keep below heart level, remove tight items. Do NOT cut/suck/tourniquet. Call 108, FRU/DH ≤30 min (anti-snake-venom). Animal bite: wound wash 15 min + ARV/RIG.
EM-007	hard_stop	 RED	Someone has swallowed pesticide / poison / an overdose.	Hard-stop. Do NOT induce vomiting. Bring the container/label. Protect airway, left-lateral if drowsy. Call 108, FRU/DH ≤30 min.
EM-008	hard_stop	 RED	A patient has cold clammy skin and a very weak pulse (shock).	Hard-stop. Lay flat, keep warm, raise legs, nothing by mouth. Call 108, FRU/DH ≤30 min.
EM-009	hard_stop	 RED	A road-accident victim with heavy bleeding from a large wound, or a severe burn.	Hard-stop. Direct pressure on bleeding; for burns cool with clean water, cover, apply nothing. Immobilise fractures. Call 108, FRU/DH ≤30 min.

 Note: PNC-001 (the postpartum-haemorrhage hard-stop) is a core life-threatening RED rule still flagged draft — worth formally signing off rather than leaving as draft.